

100-0-3

Carcinoma urothelial, NOS 8120/3
Site Code: bladder, NOS C67.9

12/30/10

for

SURGICAL PATHOLOGY REPORT**Patient Name:****Med. Rec. #:**

DOB: (Age: 57)

Gender: M

Ref. Physician:

Patient Address:

Location:

Service: Urology

Accession #:

Date of Procedure:

Date of Receipt:

Date of Report:

Account #:

Billing Type:

INPATIENT

Additional Copy to:

Ref. Source:

Clinical Diagnosis & History:

58 year old male with large volume HG bladder carcinoma.

Specimens Submitted:

- 1: SP: Rectus sheet, biopsy
- 2: SP: Urethra, margin; biopsy
- 3: SP: Bladder and prostate; radical cystoprostatectomy
- 4: SP: Lymph nodes, right pelvic; dissection
- 5: SP: Lymph nodes, left pelvic; dissection
- 6: SP: Pre-vesicle tissue, biopsy
- 7: SP: Abdominal wall hernia sac; resection
- 8: SP: Small bowel; resection
- 9: SP: Ureter, left distal; biopsy
- 10: SP: Ureter, right distal; biopsy
- 11: SP: Ureter, right distal high proximal margin; biopsy
- 12: SP: Ureter, right distal high proximal margin #2; biopsy

DIAGNOSIS:**1. SP: Rectus sheet, biopsy**

Benign fibroadipose tissue.

2. SP: Urethra, margin; biopsy

Benign prostatic urethra.

3. SP: Bladder and prostate; radical cystoprostatectomy:

a.

Tumor Type:

Invasive urothelial carcinoma, NOS

Histologic Grade:

High grade

Pattern of growth of the Non-Invasive component:

Papillary

Pattern of growth of the Invasive component:

Inverted / Nodular

Tumor Multicentricity:

Identified

Bladder Local Invasion:

Perivesical soft tissues

There is a perforation through the bladder wall with focal nests of carcinoma infiltrating through the perivesical soft tissue

 UID:33855148-B5B5-46AE-B92D-61B64D771184
 TCGA-DK-A1AA-01A-PR

Redacted



Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	MB	
Case Reviewed:	12/30/10	

SURGICAL PATHOLOGY REPORT

Extravesical Tumor Extension:

Ureters uninvolved
Urethra uninvolved

Vascular Invasion:

Not identified

Perineural Invasion:

Not identified

Surgical Margins:

Free of tumor

Non-Neoplastic Mucosa:

Exhibiting proliferative cystitis (Brunn's nests, cystitis cystica, cystitis glandularis)
There is a perforation through the bladder wall with exuberant inflammation, foreign body giant cell reaction and myofibroblastic proliferation

Prostate:

Other see part b

Seminal Vesicles:

Not identified

Perivesical Lymph Nodes:

Not identified

The Pathologic Stage is (AJCC 2002):

pT3 (Invasion of perivesicle soft tissue)

PR0 (No involvement of prostate)

b.

Tumor Type:

Adenocarcinoma

Gleason's Grade:

Primary Gleason grade:3
Secondary Gleason grade:4
Total Gleason score:7

Tumor Location:

Involves Right posterior
Involves Right anterior
Involves Left posterior
Involves Left anterior
Dominant tumor mass located in: right and left posterior apex-mid

Vascular Invasion:

Not Identified

Perineural Invasion:

Identified

Tumor Multicentricity:

Multicentric foci of invasive carcinoma are present

High Grade Prostatic Intraepithelial Neoplasia:

Identified

Capsule:

Tumor invades into, but not beyond prostate capsule

Seminal Vesicles:

Not involved

SURGICAL PATHOLOGY REPORT

Bladder Neck:
Not Involved

Surgical Margins:
Free of tumor

Non-Neoplastic Prostate:
Exhibits nodular hyperplasia

Staging (AJCC 1997):
pT2b (confined to the prostate & capsule, involving both lobes)

4. SP: Lymph nodes, right pelvic; dissection:
Five benign lymph nodes (0/5).

5. SP: Lymph nodes, left pelvic; dissection:
Eight benign lymph nodes (0/8).

6. SP: Pre-vesical tissue, biopsy:
Necrotic tissue and fibrinopurulent exudate.

7. SP: Abdominal wall hernia sac; resection:
Hernia sac.

8. SP: Small bowel; resection:
Unremarkable segment of small bowel.

9. SP: Ureter, left distal; biopsy:
Benign segment of ureter.

10. SP: Ureter, right distal; biopsy:
Benign segment of ureter.

11. SP: Ureter, right distal high proximal margin; biopsy:
Benign segment of ureter.

12. SP: Ureter, right distal high proximal margin #2; biopsy:
Benign segment of ureter.

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

*** Report Electronically Signed Out ***

Special Studies:

Result Spec	al Stain	Comment
RECUT		
AE1:AE3		
IMM	RECUT	
NEG	CONT	

SURGICAL PATHOLOGY REPORT

Gross Description:

1) The specimen is received fresh for frozen section labeled, "rectus sheet", and consists of one tan red soft tissue measuring 0.9 x 0.7 x 0.2 cm. Entirely submitted.

Summary of sections:
FSC-frozen control

2) The specimen is received fresh for frozen section labeled, "urethral margin", and consists of one tan pink soft tissue measuring 1.5 x 0.7 x 0.3 cm. Entirely submitted.

Summary of sections:
FSC-frozen control

3) The specimen is received fresh labeled, "Bladder, prostate, seminal vesicles, urethra, urachus, perivesical tissue and nodes, left rectus muscle". It consists of a cystoprostatectomy specimen with attached soft tissue at the superior aspect measuring 18.5 cm from superior to inferior, 14.5 cm laterally and 6 cm from anterior to posterior. A disrupted cyst-like structure, probably representing an organized hematoma is identified in the soft tissue, partially covered by skeletal muscle on the anterior surface. The inner surface of the structure is hemorrhagic. The skeletal muscle portion measures 6 x 2.2 x 0.3 cm. The anterior aspect of the bladder is inked black and the posterior blue; the soft tissue margin is inked green. The prostatic urethral margin is shaved and submitted. A urachal remnant measuring approximately 6 cm in length and 0.3 cm in diameter is identified. The bladder is opened along the anterior midline to reveal seven cauliflower-like fungating masses occupying most of the bladder cavity measuring from 0.5 x 0.5 x 0.5 cm to 5 x 6 x 4 cm, situated on the anterior, right and left lateral and posterior walls and dome. The stalk of the left lateral tumor is 1.6 cm from the left orifice and the stalk of the right lateral tumor is adjacent to the orifice. Both ureters are probe patent measuring up to 0.2 cm in maximum diameter. Serial sectioning reveals that the tumors are limited to the stalk and do not show gross invasion. Serial sectioning through the anterior portion /dome of the bladder reveals a perforation with a sinus tract draining into the soft tissue attached to the bladder, 0.2 cm from the hematoma. The remaining bladder mucosa adjacent to the tumors shows ulcerations. Discrete perivesical lymph nodes are not grossly identified and the specimen is submitted for lymph node dissection. The prostate is serially sectioned to reveal unremarkable prostate parenchyma. The prostate is submitted entirely. TPS is submitted, photographs are taken.

Summary of sections:

UTHM - urethral margin

RUM - right ureter margin

LUM - left ureter margin

URM - urachal margin

T1 - tumor 1 (5 x 3 x 2 cm, left lateral/anterior wall)

T2 - tumor 2 (3.5 x 3 x 2 cm, left lateral wall)

T3 - tumor 3 (0.5 x 0.5 x 0.5, posterior wall)

T4, T5 - tumors 4 and 5 (1.2 x 0.7 x 0.6; 0.6 x 0.4 x 0.3 cm; posterior wall)

T6 - tumor 6 (0.7 x 0.6 x 0.6 cm, right lateral wall)

T7 - tumor 7 (5 x 6 x 4 cm, right lateral wall)

T8 - tumor 8 (3 x 2 x 0.5 cm, right lateral wall)

T9 - tumor 9 (1.3 x 1.5 x 0.5 cm, right lateral wall)

P - perforation

TRI - trigone

RUO - right ureter orifice

LUO - left ureter orifice

RSV - right seminal vesicle

LSV - left seminal vesicle

RAP - right apex prostate

LAP - left apex prostate

RAM - right anterior mid prostate

RPM - right posterior mid prostate

LAM - left anterior mid prostate

LPM - left posterior mid prostate

RAB - right anterior base prostate

RPB - right posterior base prostate

LAB - left anterior base prostate

LPB - left posterior base prostate

ADD - additional sections of prostate

SURGICAL PATHOLOGY REPORT

4) The specimen is received in formalin, labeled "right pelvic lymph nodes" and consists of multiple pieces of fatty tissue measuring in aggregate 7 x 5 x 1.5 cm. The specimen is submitted for lymph node dissection. Lymph nodes range in size from 0.5 to 2.0 cm in greatest dimension. All identified lymph nodes are submitted.

Summary of sections:

LN-lymph nodes
BLN-bisected lymph nodes

Light

5) The specimen is received in formalin, labeled "left pelvic lymph nodes" and consists of multiple pieces of fatty tissue measuring in aggregate 5 x 4 x 1.5 cm. The specimen is submitted for lymph node dissection. Lymph nodes range in size from 0.4 to 2.0 cm in greatest dimension. All identified lymph nodes are submitted.

Summary of sections:

LN-lymph nodes
BLN-bisected lymph nodes

6) The specimen is received in formalin, labeled "Necrotic pre-vesical tissue" and consists of multiple fragments of tan necrotic soft tissue measuring 3.5 x 2 x 0.6 cm in aggregate. Representative section is submitted.

Summary of sections:

U-undesignated

7) The specimen is received in formalin, labeled "Abdominal wall hernia sac" and consists of a piece of ovoid-shaped tan fibromembranous sac measuring 3.0 x 2.5 x 2.5 cm. Cut section reveals a sac filled with yellow adipose tissue. Representative sections are submitted.

Summary of sections:

U-undesignated

8) The specimen is received in formalin, labeled "Small bowel" and consists of a piece of unoriented bowel measuring a 3.0 cm in length and 2.0 cm in diameter. Both ends of the bowel are stapled. Cut section reveals a mucosal wall thickening up to 0.7 cm. The staples are removed and representative sections submitted.

Summary of sections:

BE - both ends
RS - representative sections

9) The specimen is received in formalin, labeled "Distal left ureter" and consists of unoriented tan tubular fibroelastic soft tissue measuring 0.8 cm in length and 0.5 cm in diameter. Entirely submitted in toto.

Summary of sections:

U-undesignated

10) The specimen is received in formalin, labeled "Distal right ureter, large clip on proximal margin" and consists of an oriented tan tubular fibroelastic tissue measuring 2.5 cm in length and 0.6 cm in diameter. A clip is attached to one end. The specimen is serially sectioned and entirely submitted.

Summary of sections:

CE - clip end
OE - open end
SS - serial section, clip end to open end, remaining specimen

SURGICAL PATHOLOGY REPORT

11) The specimen is received in formalin, labeled "Distal right ureter, high proximal margin" and consists of a piece of unoriented tan tubular fibroelastic tissue measuring 0.6 cm in length and 0.3 cm in diameter. Entirely submitted in toto.

Summary of sections:

U-undesignated

12) The specimen is received in formalin, labeled "Distal right ureter, high proximal margin #2" and consists of a piece of unoriented tan tubular fibroelastic tissue measuring 0.5 cm in length and 0.4 cm in diameter. Entirely submitted in toto.

Summary of sections:

U-undesignated

Summary of Sections:

Part 1: SP: Rectus sheet, biopsy

Block	Sect. Site	PCs
1	fsc	1

Part 2: SP: Urethra, margin; biopsy

Block	Sect. Site	PCs
1	fsc	1

Part 3: SP: Bladder and prostate; radical cystoprostatectomy

Block	Sect. Site	PCs
4	ADD	4
1	LAB	1
1	LAM	1
1	LAP	1
1	LN	1
1	LPB	1
1	LPM	1
1	LSV	1
1	LUM	1
1	LUO	1
11	P	11
2	q	2
1	RAB	1
1	RAM	1
1	RAP	1
1	RPB	1
1	RPM	1
1	RSV	1
1	RUM	1
1	RUO	1
3	T1	3
1	T3	1
1	T4,5	1
1	T6	1
4	T7	4
1	T8	1
1	T9	1
1	TRI	1
1	URM	1
1	UTHM	1

Part 4: SP: Lymph nodes, right pelvic; dissection

Block	Sect. Site	PCs
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SURGICAL PATHOLOGY REPORT

3	BLN	6
1	LN	3

Part 5: SP: Lymph nodes, left pelvic; dissection

Block	Sect. Site	PCs
1	BLN	2
3	LN	6

Part 6: SP: Pre-vesicle tissue, biopsy

Block	Sect. Site	PCs
1	U	1

Part 7: SP: Abdominal wall hernia sac; resection

Block	Sect. Site	PCs
2	U	2

Part 8: SP: Small bowel; resection

Block	Sect. Site	PCs
1	BE	1
2	RS	2

Part 9: SP: Ureter, left distal; biopsy

Block	Sect. Site	PCs
1	U	1

Part 10: SP: Ureter, right distal; biopsy

Block	Sect. Site	PCs
1	CE	1
1	OE	1
2	SS	6

Part 11: SP: Ureter, right distal high proximal margin; biopsy

Block	Sect. Site	PCs
1	U	1

Part 12: SP: Ureter, right distal high proximal margin #2; biopsy

Block	Sect. Site	PCs
1	U	1

Intraoperative Consultation:

Note: The diagnoses given in this section pertain only to the tissue sample examined at the time of the intraoperative consultation.

1. FROZEN SECTION DIAGNOSIS: SP: Rectus sheet : Benign fibroadipose tissue
PERMANENT DIAGNOSIS: SAME
2. FROZEN SECTION DIAGNOSIS: SP: Urethral margin (): Benign
PERMANENT DIAGNOSIS: SAME

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIIFAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle) <u>QUALIFIED</u> / DISQUALIFIED		
Reviewer Initials		
Date Reviewed:		